CSE4203 Principles of Software Architecture

RapidMed

Emergency Triage and Real-Time Ambulance Dispatch System

Milestone 1: Proposal and architectural requirements

Task 4: Functional requirements

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Team	RapidMed
Course	CSE4203 Principles of Software Architecture
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Milestone	Milestone 1, proposal and architectural requirements
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4. Functional requirements

ID	MoSCoW priority	User story	In M3 prototype
US-01	Must	As a **field paramedic**, I want to sign in to the shared ambulance computer with my own account, so that what I record and open is attributed to me and kept apart from other crews' data.	Yes
US-02	Must	As a **field paramedic**, I want to record vital signs and a triage level in a few taps, with or without a signal, so that data entry never takes my attention from the patient or gets lost.	Yes
US-03	Must	As a **field paramedic**, I want a recommended hospital with a reserved bed within seconds of asking, so that I can leave for the right hospital without phone calls.	Yes
US-04	Must	As a **field paramedic**, I want the patient's allergies, conditions and medications on screen quickly, through break-glass if the patient cannot consent, so that I avoid giving a harmful treatment.	Yes
US-05	Must	As a **hospital triage coordinator**, I want two critical patients never to be sent for one remaining bed, so that every patient we accept has a bed waiting.	Yes
US-06	Must	As a **hospital triage coordinator**, I want crews to see the capacity I entered and how recent it is, so that they decide on information they can judge.	No
US-07	Must	As a **hospital triage coordinator**, I want early notice of each incoming patient with vital signs and arrival time, so that my team is ready when the ambulance arrives.	Yes
US-08	Should	As a **hospital triage coordinator**, I want to update capacity in seconds during a busy shift, so that the figures stay accurate without slowing my work.	No
US-09	Should	As a **hospital triage coordinator**, I want a reserved bed released automatically if the ambulance neither arrives nor renews the reservation in time, so that beds are not held for patients who went elsewhere.	No
US-10	Must	As a **central dispatch supervisor**, I want a live view of every ambulance and hospital on one map, so that I can see the whole situation in one place.	No
US-11	Must	As a **central dispatch supervisor**, I want my routing overrides to reach the crew at once and be confirmed, so that I know the crew has acted on the change.	No
US-12	Should	As a **central dispatch supervisor**, I want alerts when a crew loses its connection or a hospital fills, so that I can step in before it affects a patient.	No
US-13	Should	As a **central dispatch supervisor**, I want to review every break-glass access, so that emergency access stays justified and questions from the regulator can be answered.	No
US-14	Could	As a **central dispatch supervisor**, I want to see why a hospital was recommended, so that I can decide whether to override it.	No
US-15	Won't (this time)	As a **hospital triage coordinator**, I want bed counts read automatically from the hospital information system, so that I do not have to enter them.	No